

DYNA COLCHICINE TABLET 0.5MG

MAL19962516AZ

DESCRIPTION:

TABLET

Colour : White
Shape : Round/Biconvex
Coating : Film-coated

EACH TABLET CONTAINS: Colchicine 0.5 mg

PHARMACODYNAMICS:

Colchicine produces a dramatic response in acute gout probably by reducing the inflammatory reaction to urate crystals; this effect might be due to several actions including decreased leucocyte mobility. It is not an analgesic and has no effect on blood concentrations of Uric Acid, or on the excretion of Uric Acid. Colchicine also has an antimitotic action.

PHARMACOKINETICS:

Colchicine is readily absorbed from the gastrointestinal tract and reaches peak concentration in the plasma within 2 hours. It is partially deacetylated in the liver and the unchanged drug and its metabolites are excreted in the bile and undergo intestinal reabsorption. Colchicine is found in high concentrations in leucocytes, kidneys, the liver, and spleen. Most of the drug is excreted in the faeces but 10 to 20% is excreted in the urine and this proportion rises in patients with liver disorders.

INDICATION:

It is used for the relief of acute gout when the dramatic response can be diagnostic. It is also used for the prophylaxis of recurrent gout and to prevent acute attacks during the first few months of treatment with Allopurinol or uricosurics.

RECOMMENDED DOSE:

Treatment of acute gout attack:

1 mg (2 tablets) to start followed by 500 micrograms (1 tablet) after 1 hour.

No further tablets should be taken for 12 hours.

After 12 hours, treatment can resume if necessary with a maximum dose of 500 micrograms (1 tablet) every 8 hours until symptoms are relieved.

The course of treatment should end when symptoms are relieved or when a total of 6 mg (12 tablets) has been taken.

No more than 6 mg (12 tablets) should be taken as a course of treatment.

After completion of a course, another course should not be started for at least 3 days (72 hours).

Prophylaxis of gout attack during initiation of therapy with allopurinol and uricosuric drugs:

500 micrograms twice daily.

The treatment duration should be decided after factors such as flare frequency, gout duration and the presence and size of tophi have been assessed.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY.

CONTRAINDICATIONS:

Colchicine is contraindicated in:

- Hypersensitivity to the active substance or to any of the excipients.
- Patients with blood dyscrasias.
- Pregnancy.
- Breastfeeding.
- Women of childbearing potential unless using effective contraceptive measures.
- Patients with severe renal impairment.
- Patients with severe hepatic impairment.
- Patients undergoing haemodialysis since it cannot be removed by dialysis or exchange transfusion.
- Patients with renal or hepatic impairment who are taking a P-glycoprotein (P-gp) inhibitor or a strong CYP3A4 inhibitor.

WARNING AND PRECAUTIONS:

Colchicine is potentially toxic so it is important not to exceed the dose prescribed by a physician with the necessary knowledge and experience.

Colchicine has a narrow therapeutic window. The administration should be discontinued if toxic symptoms such as nausea, vomiting, abdominal pain, diarrhoea occur.

Colchicine may cause severe bone marrow depression (agranulocytosis, aplastic anaemia, thrombocytopenia). The change in blood counts may be gradual or very sudden. Aplastic anaemia in particular has a high mortality rate. Periodic checks of the blood picture are essential.

If patients develop signs or symptoms that could indicate a blood cell dyscrasia, such as fever, stomatitis, sore throat, prolonged bleeding, bruising or skin disorders, treatment with Colchicine should be immediately discontinued and a full haematological investigation should be conducted straight away.

Colchicine should be given with great care in case of:

- Liver or renal impairment.
- Cardiovascular disease.
- Gastrointestinal disorders.
- Elderly and debilitated patients.
- Patients with abnormalities in blood counts.

Patients with liver or renal impairment should be carefully monitored for adverse effects of Colchicine.

Co-administration with P-gp inhibitors and/or moderate or strong CYP3A4 inhibitors will increase the exposure to Colchicine, which may lead to Colchicine induced toxicity including fatalities. If treatment with a P-gp inhibitor or a moderate or strong CYP3A4 inhibitor is required in patients with normal renal and hepatic function, a reduction in Colchicine dosage or interruption of Colchicine treatment is recommended.

This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

DRUG INTERACTIONS:

- Potential risk of severe drug interactions, including death, in certain patients treated with Colchicine and concomitant P-glycoprotein or strong CYP3A4 inhibitors such as Clarithromycin, Cyclosporin, Erythromycin, Calcium Channel Antagonists (e.g. Verapamil and Diltiazem), Telithromycin, Ketoconazole, Itraconazole, HIV protease inhibitors and Nefazodone.
- P-glycoprotein or strong CYP3A4 inhibitors are not to be used in patients with renal or hepatic impairment who are taking Colchicine.
- A dose reduction or interruption of Colchicine treatment should be considered in patients with normal renal or hepatic function if treatment with a P-glycoprotein inhibitor or strong CYP3A4 inhibitor is required.
- Avoid consuming grapefruit and grapefruit juice while using Colchicine.

PREGNANCY AND LACTATION:

Use of Colchicine during pregnancy is not advised.

Chromosomal aberrations have been reported in some patients on prolonged Colchicine therapy although a causal relationship is doubtful. Controlled studies in humans have not been performed. The data available do not establish specific teratogenic effects. It is not known whether Colchicine is distributed into human milk and it should therefore be used with caution in nursing women.

SIDE EFFECTS:

The most frequent adverse effects of Colchicine are those involving the gastrointestinal tract and may be associated with its antimitotic action. Diarrhoea, nausea, vomiting, and abdominal pain are often the first signs of toxicity and are usually an indication that Colchicine therapy should be stopped. Bone marrow depression with agranulocytosis thrombocytopenia, and aplastic anaemia have occurred on prolonged treatment as have peripheral neuritis, myopathy rashes and alopecia.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdose may cause burning feeling in stomach, throat or skin; vomiting; convulsions; diarrhoea, fever, muscle weakness; pulmonary edema; respiratory depression or renal failure.

Colchicine should be withdrawn or the dose reduced if adverse gastrointestinal effects occur.

For treatment in acute poisoning, the stomach should be emptied by aspiration and lavage. A purgative such as Sodium Sulphate 30 g in 250 ml of water should be given. Demulcent drinks may be given freely. Morphine Sulphate, 10 mg, intramuscularly may be given to relieve abdominal cramps. Atropine has also been given.

PACKING/PACK SIZE(S):

Plastic container of 100's.

Blister pack of 10x10's.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)
2497, Mk. 1, Lorong Perusahaan Baru 5,
Kawasan Perusahaan Perai 3,
13600 Perai, Pulau Pinang, Malaysia.

PI1PT D066-01
Latest Revision: Nov 2020